

ACGME Common Program Requirements (Residency) – Major Revision

Response Guide for USF Emergency Medicine Faculty and Residents

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Comment deadline: October 22, 2026 • Form: acgme.qualtrics.com/jfe/form/SV_7NEm7TaI8AQBSy •

Documents: acgme.org → Review and Comment → Common Program Requirements (Residency)

Summary: what Dr. Wilson submitted (October 8, 2026)

The selections below are what was entered on the ACGME form under Dr. Wilson's name. Use this page as the quick reference; the page-by-page tables that follow give the exact comment text for each row. RC = Review Committee for Emergency Medicine. PD = program director.

- **Section 1 (1.1–1.12):** 1.1–1.5 left blank; 1.6–1.12 Support as proposed.
- **2.1–2.10:** 2.1–2.7.a and 2.10 Support; 2.8 Support with modifications (RC should confirm PD qualifications before appointment takes effect); 2.8.a Do not support (keep RC final approval of PD); 2.9 Do not support (keep PD continuity requirement).
- **2.11–2.14:** 2.11, 2.11.a, 2.12, 2.13 Do not support with comments (three-year floor, mentoring plan is not a substitute, PD must be ABEM/AOBEM certified, keep RC authority over PD qualifications); 2.14 Support with comment.
- **2.15–2.22:** 2.15–2.15.l blank. 2.16–2.21.d Support. 2.22 Do not support (supervision “flexibility” for other specialties).
- **2.23–2.25.a:** 2.23 Support; 2.24, 2.24.a, 2.25 Do not support (faculty board certification; 1:5 core faculty cap is the most damaging change, 13→8 at USF); 2.25.a Support with modifications (protected time).
- **2.26–3.7:** Support throughout.
- **4.1–4.3.h:** Support; comment on 4.3 (AI/ethics language welcome).
- **4.4, 4.5, 4.6:** Do not support (outcome→process language; RC must keep authority over patient care, procedures, medical knowledge).
- **4.7, 4.8, 4.9:** Support with modifications (keep “residents must demonstrate” language and RC authority); sub-items blank.
- **4.10–4.15:** 4.10–4.14 Support. 4.15 Do not support (pain management/SUD should stay a common requirement).
- **4.16–4.17.a:** 4.16–4.16.b blank. 4.17–4.17.a Support, with comment endorsing environment of inquiry.
- **4.18–4.19:** 4.18, 4.18.a, 4.19 Do not support; 4.18.b, 4.18.c Support with modifications (faculty scholarship floor too low; residents need scholarly product plus patient safety/QI participation).

- **5.1–5.16.e:** 5.1–5.14.h Support. 5.15 Support with modifications (keep pooled three-year first-time rate; 2-of-3 forgives a failed cohort). 5.16 Support with modifications (90% ultimate standard, measured against all graduates, paired with a first-time standard that bites). 5.16.a–e Support.
- **6.1–6.26:** 6.1–6.4, 6.6–6.26 Support. 6.5 Support with modifications (real patient safety reviews/QI participation).
- **6.27–6.28.b:** Blank.
- **6.29–6.35.a:** 6.29 Support with modifications (80-hr ceiling fine; RC must keep ability to set 60). 6.30 Do not support (deleting 8 hours off leaves no turnaround minimum for EM). 6.31 Support. 6.32, 6.33 Support with modifications (one-in-seven without averaging; 12-hour shift limit). 6.33.a–6.35.a Support.
- **6.36–6.39.a:** 6.36–6.39 Support. 6.39.a Do not support (deleting the “[RC may further specify under 6.29–6.39]” bracket).

Note: the form has no general comment box at the end, and the Next page button on the final comment page (6.39.a) submits the survey. There is no review screen and no confirmation email, so keep your own record of what you entered.

Why this matters

The ACGME is proposing a major revision of the Common Program Requirements that apply to every residency. The ACGME presents it as a 43 percent reduction in the number of requirements. For Emergency Medicine the effect is different: in several areas it deletes existing EM-specific standards and removes or narrows the EM Review Committee’s authority to put them back. The proposal goes to the ACGME Board in June 2027 with an effective date of July 1, 2028. The public comment period closes October 22, 2026. Individual comments are counted; a department-level letter is read once.

Three themes run through every answer in this guide. If you write anything in your own words, write to these:

- **1. Standards.** The ACGME’s job is to guarantee that every accredited program produces a highly trained physician for the community it serves. Requirements that are easier to meet are not the same as requirements that are better for patients or residents.
- **2. Specialty control.** The ACGME should set the floor and let each specialty, through its Review Committee, adapt, modify, and adopt the requirements that fit its field. Emergency Medicine should be able to keep a 60-hour week, a 12-hour shift, a 1:3 core faculty ratio, and EM-trained supervision because those standards fit emergency care.
- **3. Balance against corporate influence.** GME expansion in EM over the last decade has been dominated by programs outside academic institutions. Corporate and private-equity-backed employers see less regulation and lower cost as favorable. The ACGME should be the counterweight, not the accelerant.

The six changes that hit EM hardest

- **Core faculty ratio (2.25)** — Caps what any specialty may require at 1 core faculty : 5 residents. EM is 1:3. For USF EM that is 13 required core faculty down to 8.
- **Program director qualifications (2.11–2.13)** — Three-year experience minimum replaced by a mentoring plan; non-board-eligible PD can be approved by the GMEC; EM-RRC loses authority to require ABEM/AOBEM or add qualifications.
- **Faculty qualifications and supervision (2.22, 2.24)** — GMEC pathway for non-board-eligible faculty; RRC authority to require ABMS/AOA deleted; specialties told their supervision rules “should include flexibility” for other-specialty faculty.

- **Patient care, procedures, medical knowledge (4.4–4.6)** — “Residents must demonstrate” becomes “programs must provide instruction in and assess”; RRC authority to specify EM competencies deleted.
- **Scholarship (4.18–4.19)** — Faculty minimum becomes one activity every three years (one lecture qualifies); RRC can no longer require peer-reviewed publication or specify resident scholarship.
- **Work hours (6.29–6.39)** — 80-hour ceiling is fine; the problems are deleting the 8-hours-off turnaround rule (6.30) and deleting RRC authority to keep EM’s 60-hour week, 12-hour shifts, and non-averaged day off.

How the comment form works

- Open the form link above in a normal (not private) browser window. You can close the browser and come back later on the same device; progress is saved.
- Pages 1–3 are demographics (name, role, institution, specialty). Page 4 asks whether you support everything with no comments or want to continue. Choose CONTINUE.
- The rest of the form is one page per section of the requirements (about 35 pages). Each page shows the tracked-changes text, then a grid with one row per requirement number and four checkboxes: Support as proposed / Support with additional modifications / Do not support / I wish to include a comment.
- Nothing is required. You can leave any row blank. Checking “I wish to include a comment” does not open a box on the same page. The comment boxes for the rows you checked (“Comments on 2.8.” etc., 1,600-character limit) appear on the next page after you click Next page. That is where the comment text in this guide goes.
- Click Next page to move on. Clicking Next too quickly can trigger a “page is out of date” dialog; choose Refresh and re-enter that page. There is no separate Submit button: Next page on the last comment page (6.39.a) submits the survey and shows a thank-you page.
- Following this guide takes 20–30 minutes. Pages marked PRIORITY below are the ones to spend time on; the rest can be left blank or marked Support.
- Identical pasted comments from many people are weighted as one. Keep the position and the selection, but change a sentence or add one of your own (“As a PGY-2...”, “As core faculty who runs our ultrasound curriculum...”). The ACGME has said it will not reply to individual comments.

Pages 1–4: Demographics and the continue question

Question on the form	Dr. Wilson’s answer	What you should enter
First Name / Last Name / Credentials / Email	Jason / Wilson / MD, PhD / (USF email)	Your own. Use your institutional email.
What is your current role?	Department Chair	Residents: “Resident.” Core faculty: “Faculty Member, Core.” Other faculty: “Faculty Member, Non-Core.” PD/APDs: “Program Director/Associate Program Director (Residency).” Fellowship directors: “...(Fellowship).” Fellows: “Fellow.”
Are you a recent graduate (completed GME within the past five years)?	No	Yes if you finished residency or fellowship in 2021 or later. The ACGME reads recent-graduate responses as a

Question on the form	Dr. Wilson's answer	What you should enter
		distinct voice, so answer accurately.
What best describes your current institution type?	Academic Medical Center/Medical School	Same for all USF EM faculty and residents.
What best describes your current work setting?	Urban	Same for all.
How many ACGME-accredited programs does your Sponsoring Institution oversee?	76–100	USF Morsani College of Medicine is the Sponsoring Institution. Select 76–100 (or “I don’t know” if unsure).
How many residents/fellows are currently in your ACGME-accredited program(s)?	31–50	USF EM residency (expanding to 13 per year). Select 31–50.
Select your specialty(ies)/subspecialty(ies)	Emergency Medicine (also EM Subspecialty)	Check “Emergency Medicine.” Add “Emergency Medicine Subspecialty” if you work in or direct a fellowship (EMS, Toxicology, etc.).
Please select from one of the following statements	I would like to continue this survey and provide comments on the proposed Common Program Requirements.	Do NOT pick the first option — it records blanket support and ends the survey.

Requirement pages: selection and comment text

Columns: the requirement row(s) as they appear in the grid, the box to check, and the text to paste after checking “I wish to include a comment.” Where the comment cell is empty, check the box and move on. Where the selection says “Leave blank,” the row is a relocation note with nothing to evaluate.

Key: **S** Support as proposed **M** Support with additional modifications **D** Do not support – Leave blank

Page 5: Section 1: Oversight – Sponsoring Institution (1.1–1.5)

Housekeeping moves (PLA and site-director language relocated to 1.6). Nothing here affects EM. Leave blank and click Next page, or check “Support as proposed” on each row.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
1.1 – 1.5	– Leave blank	—

Page 6: Section 1: Oversight – Participating Sites (1.6–1.6.d)

Site changes move from Review Committee approval to GMEC approval. Reasonable; no comment needed.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
1.6 – 1.6.d	S Support as proposed	—

Page 7: Section 1: Oversight – Required Resources and Well-Being Environment (1.7–1.10)

Deletes the generic resources statement (RC still specifies EM resources) and expands lactation, pregnancy, and fatigue provisions. Supportable.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
1.7 – 1.10	S Support as proposed	—

Page 8: Section 1: Oversight – Other Learners and Health Care Personnel (1.11–1.12)

Splits one requirement into two (other learners; APPs). Supportable.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
1.11 – 1.12	S Support as proposed	—

PRIORITY Page 9: Section 2: Personnel – Program Director (2.1–2.10)

Most rows are reorganization. Two items matter: 2.8/2.8.a removes Review Committee approval of a new program director; 2.9 deletes the program-director continuity requirement.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
2.1 – 2.7.a	S Support as proposed	—
2.8	M Support with additional modifications	GMEC verification of licensure and appointment is reasonable, but the Review Committee should retain the ability to confirm that a new program director meets the specialty’s certification and experience qualifications before the appointment takes effect. Citing a program after the fact for an unqualified program director puts residents, not the sponsoring institution, at risk.
2.8.a	D Do not support	Do not delete Review Committee final approval of the program director. The Review Committee for Emergency Medicine has the specialty-specific knowledge to judge whether a candidate is qualified

Row(s)	Check	Comment text (paste after checking "I wish to include a comment")
		to lead an EM residency; a GMEC does not.
2.9	D Do not support	Program director continuity is directly tied to program stability and the resident experience. Frequent turnover is a recognized warning sign, and deleting this requirement removes one of the few early signals the Review Committee has. If the concern is penalizing programs for departures beyond their control, narrow the requirement rather than delete it.
2.10	S Support as proposed	—

PRIORITY Page 10: Section 2: Personnel – Qualifications of the Program Director (2.11–2.14)

This page loosens who can be a program director: the three-year experience minimum is replaced by a mentoring plan, a non-board-eligible PD can be approved by the GMEC, and the Review Committee loses the authority to require ABMS/AOA certification or to add PD qualifications.

Row(s)	Check	Comment text (paste after checking "I wish to include a comment")
2.11	D Do not support	Three years is not a magic number, but it is a reasonable floor. The program director is the single most important determinant of residency quality. The role should be filled by an experienced educational leader, not by the most available faculty member supported by a mentoring plan. A program that cannot field an experienced, expert-level program director should not be a program. Retain the three-year minimum and keep the exception pathway with the Review Committee, which understands the specialty's needs.
2.11.a	D Do not support	A mentoring plan is a good practice for every new program director; it is not a substitute for the experience requirement. If retained, make it an addition to 2.11, not an alternative to it.
2.12	D Do not support	The program director must be a board-certified expert in the specialty of the program. In Emergency Medicine that means current ABEM or AOBEM certification. A GMEC-approved pathway for an individual who is not eligible for certification in the specialty undermines the specialty's own definition of expertise, and the sponsoring institution does not have the specialty-specific knowledge to make that judgment. The Review Committee must retain the authority to specify that only ABMS/AOA certification in the specialty is acceptable.
2.13	D Do not support	Do not delete the Review Committee's authority to specify additional program director qualifications. EM currently requires ongoing clinical activity, core faculty status, and demonstrated leadership experience; those reflect the realities of running an emergency medicine residency and should remain within the specialty's control.
2.14	S Support as proposed	Supported. Protected leadership time should scale with program size and complexity, and sponsoring institutions should not treat the minimum as the ceiling.

Page 11: Section 2: Personnel – Program Director Responsibilities (2.15–2.15.I)

Every row on this page is a “moved to” note; the content survives under 2.1–2.7 and 6.17. Leave blank or check “Support as proposed.”

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
2.15 – 2.15.I	– Leave blank	—

PRIORITY Page 12: Section 2: Personnel – Faculty and Faculty Responsibilities (2.16–2.22)

The important text is the bracket at the bottom of the page, attached to 2.22: specialty requirements for same-specialty supervision “should include flexibility” for supervision by faculty in other specialties.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
2.16 – 2.21.d	S Support as proposed	—
2.22	D Do not support	I support faculty development in CBME. I do not support the bracketed instruction that specialty-specific supervision requirements “should include flexibility” for supervision by faculty in other specialties. Emergency Medicine residents caring for emergency department patients must be supervised by emergency physicians who completed an EM residency. The current EM requirements already contain defined exceptions for pediatric and low-acuity settings. This language reads as an accommodation for programs that lack sufficient EM faculty, not as a benefit to residents or patients. The Review Committee should retain full authority over supervising-faculty qualifications.

PRIORITY Page 13: Section 2: Personnel – Faculty Qualifications, Physician Faculty, and Core Faculty (2.23–2.25.a)

THE MOST IMPORTANT PAGE IN THE SURVEY. 2.24 opens a GMEC pathway for non-board-eligible faculty and deletes the Review Committee’s ability to require ABMS/AOA certification. 2.25 caps the core faculty ratio any specialty may require at 1:5 — EM is currently 1:3.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
2.23	S Support as proposed	—
2.24	D Do not support	Physician faculty who supervise and teach EM residents must be board certified in Emergency Medicine (ABEM/AOBEM). The GMEC exception process, combined with deleting the Review Committee’s authority to require only ABMS/AOA certification, lowers the faculty qualification standard. Faculty must also be scholars in the field — producing peer-reviewed work and pursuing research and service. The Review Committee, not the GMEC, is the appropriate body to define faculty qualifications for the specialty.
2.24.a	D Do not support	Same objection as 2.24: retain Review Committee authority over physician faculty qualifications.
2.25	D Do not support	This is the most damaging change in the proposal for Emergency Medicine. EM currently requires one core faculty member for every three residents. Capping what a Review Committee may require at 1:5 would cut our program’s required core faculty from 13 to 8. Core faculty status is how an academic department gives protected time to physicians who want to build careers in education and scholarship; with mentorship and career development, those faculty grow into the

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
		academicians who write the papers, run the trials, lead quality work, and train the next generation. Over the past decade, GME expansion in EM has been dominated by programs outside academic institutions, with smaller academic faculty groups that often support only the required FTE. A 1:5 ceiling will reduce dedicated faculty for didactics, mentorship, and assessment; reduce the number of emergency physicians who develop as academicians; and accelerate divestment from academic Emergency Medicine. The stated rationale — reducing variability across specialties and lowering barriers for small programs — is administrative, not educational. The Review Committee must retain the authority to set the ratio the specialty needs. If a ceiling is retained at all, it must not override a specialty’s existing, more rigorous standard.
2.25.a	M Support with additional modifications	Retain and strengthen the Review Committee’s authority to specify dedicated, protected non-clinical time for core faculty. Without protected time the core faculty designation is a label, not a resource for residents.

Page 14: Section 2: Personnel – GME Administrative Professionals (2.26–2.27)

Renames “program coordinator” and ties support to program size. Supportable.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
2.26 – 2.27	S Support as proposed	—

Page 15: Section 3: Resident Appointments (3.1–3.4.c.2)

Eligibility and transfer language, mostly reorganized. No EM-specific concern.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
3.1 – 3.4.c.2	S Support as proposed	—

Page 16: Section 3: Resident Appointments – Resident Complement (3.5–3.7)

No concern.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
3.5 – 3.7	S Support as proposed	—

Page 17: Section 4: Educational Program – Length of Program and Educational Components (4.1–4.2.e)

4.2.e (patient safety education) is deleted as redundant with Section 6. Supportable.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
4.1 – 4.2.e	S Support as proposed	—

Page 18: Section 4: Educational Program – Professionalism (4.3–4.3.h)

Adds ethical use of technology including AI. Supportable.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
4.3 – 4.3.h	S Support as proposed	Supported, including the explicit expectation of ethical use of technology and artificial intelligence.

PRIORITY Page 19: Section 4: Educational Program – Patient Care, Procedural Skills, and Medical Knowledge (4.4–4.6)

Two changes on one page: (1) the standard shifts from what residents must demonstrate to what programs must provide instruction in and assess; (2) the Review Committee’s authority to specify patient care, procedural skills, and medical knowledge is deleted, and existing EM-specific requirements in these sections will be removed.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
4.4	D Do not support	Two problems. First, the language moves from what residents must demonstrate to what programs must provide instruction in and assess — an outcome standard becomes a process standard. Second, deleting the Review Committee’s authority to further specify patient care removes the specialty’s ability to define what an emergency physician must be able to do. Graduates of every accredited EM program should have comparable ability at the end of training; that requires specialty-wide standards, not local program expectations, which are not equivalent.
4.5	D Do not support	Procedural competence (airway management, resuscitation, procedural sedation, and the rest of the EM procedure list) is the core of emergency medicine training. The Review Committee must retain authority to specify the procedures that define the specialty and the expectation that residents demonstrate competence in them.
4.6	D Do not support	Retain Review Committee authority to specify the medical knowledge base of the specialty and the expectation that residents demonstrate it. The common requirement is too general to define any specialty’s knowledge standard.

Page 20: Section 4: Educational Program – Practice-Based Learning and Improvement (4.7–4.7.f)

Sub-competencies consolidated; Review Committee authority to add sub-competencies deleted. Support the consolidation, ask that specialty authority be retained.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
4.7	M Support with additional modifications	Consolidation is reasonable, but retain the resident-level outcome language (“residents must demonstrate competence in...”) and the Review Committee’s authority to add specialty-specific sub-competencies.
4.7.a – 4.7.f	– Leave blank	—

Page 21: Section 4: Educational Program – Interpersonal and Communication Skills (4.8–4.8.g)

Same pattern as 4.7.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
4.8	M Support with additional modifications	Same comment as 4.7: support consolidation; retain resident-level outcome language and Review Committee authority to add specialty-specific sub-competencies. Including hand-offs here is appropriate.
4.8.a – 4.8.g	– Leave blank	—

Page 22: Section 4: Educational Program – Systems-Based Practice (4.9–4.9.h)

Same pattern as 4.7; adds use of emerging technology including AI.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
4.9	M Support with additional modifications	Same comment as 4.7 and 4.8. The addition of existing and emerging technologies, including artificial intelligence, is welcome.
4.9.a – 4.9.h	– Leave blank	—

Page 23: Section 4: Educational Program – Curriculum Organization and Resident Experiences (4.10–4.13)

Transitions of care and clinical workload relocated here. Supportable.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
4.10 – 4.13	S Support as proposed	—

PRIORITY Page 24: Section 4: Educational Program – Planned Educational Activities (4.14–4.15)

4.15 deletes the common requirement for instruction in pain management and recognition of substance use disorder, leaving it to Review Committees to specify.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
4.14	S Support as proposed	—
4.15	D Do not support	Pain management and recognition of substance use disorder should remain a common requirement for every specialty with patient contact, not an item a Review Committee may optionally specify. The opioid crisis touches every specialty, and the common requirement was one of the few explicit accreditation levers supporting that education.

Page 25: Section 4: Educational Program – Inquiry and Scholarship, Environment of Inquiry (4.16–4.17.a)

4.16 folds into 4.18 (next page); 4.17 adds a useful “environment of inquiry” expectation. Support 4.17; hold your comments for 4.18.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
4.16 – 4.16.b	– Leave blank	—
4.17 – 4.17.a	S Support as proposed	Supported. An explicit environment-of-inquiry requirement is a strength of the proposal and should be retained.

PRIORITY Page 26: Section 4: Educational Program – Scholarly Activity (4.18–4.19)

SECOND-MOST IMPORTANT PAGE. Faculty scholarship minimum becomes one qualifying activity every three years (a single didactic lecture satisfies it); the Review Committee can no longer require peer-reviewed publication for faculty or specify resident scholarship beyond one activity before graduation.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
4.18	D Do not support	The proposed minimum — one qualifying activity every three years, which a single didactic lecture satisfies — is not a scholarship standard. Physician faculty should be scholars in their field: producing peer-reviewed publications and pursuing research and service. The current EM requirement (accomplishment across at least three scholarly domains plus dissemination including peer-reviewed publication) is appropriate and should be preserved. Lowering the floor this far diminishes expectations for generating and disseminating new knowledge across EM and the broader house of medicine. Programs that choose to exceed the floor will be the ones with the resources to do so, widening the gap between academic and non-academic programs and producing a more tiered distribution of training quality.
4.18.a	D Do not support	Once every three years is too infrequent, and the list of qualifying activities is too broad for a minimum. Core faculty should demonstrate scholarly activity annually, and the Review Committee should be able to require peer-reviewed publication across the faculty.
4.18.b	M Support with additional modifications	Residents should complete a scholarly product before graduation and should also participate in patient safety reviews and quality improvement projects, which will be integral to their careers whether in academics or community practice. Make both elements required rather than allowing a single didactic presentation to satisfy the requirement. Residents should be trained for potential academic careers and as lifelong learners, with program support for scholarly activity.
4.18.c	M Support with additional modifications	The Review Committee for Emergency Medicine should be able to require both bullets, including peer-reviewed publication. Make explicit that Review Committees retain this authority and that the choice is not limited to the first bullet alone.
4.19	D Do not support	Do not delete the Review Committee’s authority to further specify resident scholarly activity.

Page 27: Section 5: Resident Assessment (5.1–5.3.b.2) and (5.4–5.10)

Two pages. CCC and assessment requirements are consolidated without lowering them. Supportable.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
5.1 – 5.10	S Support as proposed	—

Page 28: Section 5: Final Resident Assessment (5.11–5.12.e) and Faculty Assessment (5.13–5.13.d)

Two pages. Reorganization only.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
5.11 – 5.13.d	S Support as proposed	—

Page 29: Section 5: Program Evaluation and Improvement (5.14–5.14.h)

5.14.h deletes the Self-Study requirement. This is a burden reduction with no clear educational cost; support or leave blank.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
5.14 – 5.14.h	S Support as proposed	—

PRIORITY Page 30: Section 5: Board Certification (5.15–5.16.e)

Two changes. The 90% ultimate certification rate within seven years is new and worth supporting. But the first-time pass standard changes from a pooled three-year rate to “two of the last three administrations,” which the ACGME says is to protect small programs from one or two failures. That forgives a failed cohort, including a new program’s first graduating class. The 80% safe harbor already exists today, so that part is not a tightening.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
5.15	M Support with additional modifications	I do not support replacing the pooled three-year first-time pass rate with two of the last three administrations. The stated purpose is to protect small programs from one or two failures, but a program that fails a cohort should answer for it, not be insulated by its size. A new program’s first graduating class is the cohort most likely to fail, and this change forgives exactly that year. Keep the pooled three-year standard.
5.16	M Support with additional modifications	I support a 90 percent ultimate certification standard. Two modifications: measure it against all graduates, not only those who seek certification, and recognize that a seven-year lookback gives a new program roughly a decade before this standard can touch it, so it must sit alongside a first-time standard that bites. Outcome standards are the right way to hold programs accountable, and weakening the input standards elsewhere in this proposal (core faculty, faculty qualifications, scholarship) works against this one.
5.16.a – 5.16.e	S Support as proposed	—

Page 31: Section 6: Patient Safety and Quality (6.1–6.6)

Consolidated. Use 6.5 to make the point that residents should participate in real patient safety reviews and QI.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
6.1 – 6.4	S Support as proposed	—
6.5	M Support with additional modifications	Resident participation in real (not only simulated) interprofessional patient safety reviews and quality improvement activities should be required, with program support. This is integral to every physician’s career, academic or community.
6.6	S Support as proposed	—

Page 32: Section 6: Supervision and Accountability (6.7) and Levels of Supervision (6.8–6.10.d; 6.11–6.15)

Three pages. Supervision definitions consolidated into a required program policy; Review Committee keeps authority to specify activities requiring direct supervision. Supportable.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
6.7 – 6.15	S Support as proposed	—

Page 33: Section 6: Learning and Working Environment (6.16–6.17.d) and Well-Being (6.18–6.21.b; 6.22–6.26)

Three pages. Professionalism, psychological safety, and well-being consolidated. Supportable.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
6.16 – 6.26	S Support as proposed	—

Page 34: Section 6: Teamwork and Transitions of Care (6.27–6.28.b)

All moved to 6.4, 4.10, and 4.8. Leave blank.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
6.27 – 6.28.b	– Leave blank	—

PRIORITY Page 35: Section 6: Clinical and Educational Work Hours (6.29–6.35.a)

The 80-hour ceiling and four-week averaging are fine as universal maximums. Two things matter for EM: 6.30 (eight hours off between work periods) is deleted, and on the NEXT page (under 6.39.a) the Review Committee’s authority to further specify anything in 6.29–6.39 is deleted. 6.31 and 6.38 concern 24-hour in-house call and do not touch ED shifts. Use this page to say the common ceiling is acceptable AND that EM must keep its own limits.

Row(s)	Check	Comment text (paste after checking “I wish to include a comment”)
6.29	M Support with additional modifications	An 80-hour ceiling averaged over four weeks is acceptable as a universal maximum, and flexibility at the block level has real value for scheduling. No EM program schedules residents near that ceiling; typical scheduled ED hours are closer to 45 per week. However, the Review Committee must retain the ability to set specialty-specific limits below the ceiling. Emergency Medicine’s current standards — no more than 60 scheduled ED hours per week, 12-hour shifts, equivalent time off between shifts, no more than six consecutive days, and a 24-hour period off every seven days without averaging — reflect the cognitive load of shift-based emergency care and protect patients. A specialty should be able to decide to keep 60.
6.30	D Do not support	Do not delete the eight-hours-off minimum between scheduled work periods. Combined with the deletion of Emergency Medicine’s own “equivalent time off between scheduled ED work periods” requirement, this leaves no minimum turnaround between shifts anywhere in the requirements. Shift-based specialties depend on this protection.
6.31	S Support as proposed	—
6.32	M Support with additional modifications	Retain the Review Committee’s ability to require one day in seven free of clinical work without four-week averaging, as Emergency Medicine does today.

Row(s)	Check	Comment text (paste after checking "I wish to include a comment")
6.33	M Support with additional modifications	A 24-hour maximum is appropriate as a universal ceiling, but the Review Committee must retain the ability to set a shorter shift limit; Emergency Medicine's 12-hour limit should stand.
6.33.a	S Support as proposed	—
6.34 – 6.35.a	S Support as proposed	—

PRIORITY Page 36: Section 6: Moonlighting and At-Home Call (6.36–6.39.a) – LAST PAGE

The bracket "[The Review Committee may further specify under any requirement in 6.29–6.39]" is struck through under 6.39.a. That deletion is what removes EM's 60-hour week and 12-hour shift. Object here.

Row(s)	Check	Comment text (paste after checking "I wish to include a comment")
6.36 – 6.36.b	S Support as proposed	—
6.37	S Support as proposed	—
6.38	S Support as proposed	—
6.39	S Support as proposed	—
6.39.a	D Do not support	I do not object to the at-home call language. I object to deleting the bracket "[The Review Committee may further specify under any requirement in 6.29–6.39]." That bracket is the mechanism that lets Emergency Medicine keep its shift-length and weekly-hour limits. Deleting it removes the specialty's ability to set any work-hour standard stricter than the common ceiling. The rationale offered — consistency across specialties and institutional scheduling burden — is administrative, not educational. The ACGME should set the floor and allow each specialty to adapt the requirements to the realities of its field. Retain Review Committee authority to further specify under 6.29–6.39.

General statement (the form has no general comment box)

This form ends without an open text box. Use the statement below, after replacing [role] with your own (resident, core faculty member, program director, etc.), as additional text in the comment on 2.25 (core faculty), or as the body of an email to the ACGME.

I am commenting as [role] in Emergency Medicine at the University of South Florida Morsani College of Medicine and Tampa General Hospital. I support the goal of a simpler, less prescriptive set of Common Program Requirements, and I support the new ultimate board-certification standard. I do not support the parts of this proposal that lower specialty standards and remove Review Committee authority to restore them: the 1:5 core faculty ceiling (2.25), the GMEC pathways and loss of Review Committee authority over program director and faculty certification (2.12, 2.24), the cross-specialty supervision language (2.22), the deletion of specialty authority over patient care, procedural skills, and medical knowledge (4.4–4.6), the scholarship minimum that a single lecture can satisfy (4.18), and the deletion of Review Committee authority over work hours (6.29–6.39). Over the last decade, rapid expansion of Emergency Medicine programs outside academic institutions has already created a heterogeneous training environment in which graduates of accredited programs do not have equal ability. These changes would widen that gap and produce a more tiered distribution of programs, because only departments with resources will choose to exceed the floor. The ACGME's role should be to (1) ensure standards across programs that produce highly trained physicians for our communities, (2) allow each specialty to adapt and adopt the requirements that fit its field, and (3) act as a counterweight to corporate and private-equity influence in GME, which sees lower cost and less regulation as favorable. Protecting the training of residents and the future of academic medicine is the point of accreditation.

After you submit

- Reply to the department email (or tell your PD/APD) that you submitted, so we can track our count. Numbers matter in this process.
- Residents: CORD and EMRA are also collecting feedback; the CORD summary graphic that circulated this week is accurate and worth sharing with co-residents at other programs.
- Faculty with ACEP, SAEM, AAEM, or CORD leadership roles: ask those organizations to submit organizational responses on 2.25, 2.12/2.24, 4.4–4.6, 4.18, and 6.39.a specifically.

Sources: ACGME Proposed Major Revision of the Common Program Requirements (Residency), tracked-changes copy dated 09/08/2026; ACGME Rationale and Impact Statement, 09/08/2026; ACGME Review and Comment page; CORD educational summary "Proposed ACGME Common Program Requirement Changes: What Could They Mean for Emergency Medicine?" (October 2026); 2028 ACGME Program Requirements for Emergency Medicine as summarized by CORD.